CLINIC LAB REPORT

Official Diagnostic Results

Date: 26-May-2026

Lab Provider: Internal Setup

PATIENT NAME	PRESCRIBING DOCTOR	TEST REQUESTED
Aswin (2026-PT-33)	**Deepak**	**Blood Test**

Diagnostic Parameters

Parameter / Test	Result Value	Unit	Ref. Range
Hemoglobin	**61**	g/dL	13.0 - 17.0
WBC Count	**64**	cells/mcL	4,500 - 11,000
Platelet Count	**652**	/mcL	150,000 - 450,000

Doctor/Technician Notes

Medical Interpretation of Blood Test Results

Date of Report: [Insert Current Date]

Findings:
The following parameters are outside the provided reference ranges:
*   **Hemoglobin:** 61 g/dL (Reference Range: 13.0 - 17.0 g/dL) - **Extremely high.**
*   **WBC Count:** 64 cells/mcL (Reference Range: 4,500 - 11,000 cells/mcL) - **Critically low.**
*   **Platelet Count:** 652 /mcL (Reference Range: 150,000 - 450,000 /mcL) - **Critically low.**

Qualitative Summary:
This blood test reveals critically low white blood cell and platelet counts, alongside an extraordinarily elevated hemoglobin level.

Potential Clinical Significance:
The hemoglobin value of 61 g/dL is significantly outside physiological compatibility and strongly suggests a potential laboratory error, sample issue (e.g., severe hemoconcentration, specific analytical interference), or data transcription error. Such an extreme elevation in hemoglobin is implausible and would typically require immediate verification.
The critically low WBC count of 64 cells/mcL represents severe leukopenia, indicative of profound immunosuppression (agranulocytosis), which places the patient at an extremely high risk for severe, life-threatening infections.
The critically low platelet count of 652 /mcL signifies severe thrombocytopenia, increasing the risk of spontaneous bleeding, including serious internal hemorrhages.
The combination of critically low WBC and platelet counts with an implausibly high hemoglobin level presents a complex and concerning picture. While the severe leukopenia and thrombocytopenia point towards significant bone marrow dysfunction, severe myelosuppression, or increased peripheral destruction, the hemoglobin result necessitates immediate re-evaluation and verification. This overall clinical scenario requires urgent medical attention, with particular emphasis on confirming the hemoglobin value through repeat testing and comprehensive clinical correlation.

Disclaimer: This is an AI-generated draft intended for professional review and should be integrated with the patient's complete clinical presentation for accurate diagnosis and management.

Authorized Signature

LAB DEPARTMENT